From: Nicholas Scarr <nscarr@quantlogic.com>
To: Andy Lyne <Andy.Lyne@qlaims.com>
CC: Claims <Claims@qlaims.com>, Louisa Scarr <scarrlouisa@gmail.com>
Date: Tuesday 04 November 2025 at 19:15
Subject: Detailed understanding of the Biotechnical position relating to 1 Riverside Cottages, The Dean, Alresford SO24 9BE

Dear Darren,

Further to our meeting at BioResponse's offices last Thursday, which I attended in person (with *Andy Lyne* joining by telephone), I would like to record my understanding of the factual and professional points discussed in relation to *1 Riverside Cottages*.

Please review the summary below and confirm that it accords with BioResponse's professional understanding. If you consider any part inaccurate, please identify it and set out the technical basis for your view.

Your firm indicated from the outset that this incident needed to be dealt with *expeditiously to avoid significant secondary damage*. Alas, this did not happen, *amplifying the extent of secondary damage*.

The delay did not alter the fundamental classification of the incident — *sewage
ingress is Category 3 from the moment of contact* — but it *did allow extensive humidity and cross-contamination to develop*, exacerbating the depth of microbial infiltration. The property has remained, and continues to remain, in a contaminated condition pending comprehensive remediation and independent post-remediation verification, and is at present certainly far from any standard that could be described as hospital-grade.

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1. Chronology and Extent of Contamination

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The primary sewage ingress occurred on *28 February 2025*, affecting both the *interior of the dwelling* and the *front and rear external areas*.

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The property remained in a contaminated condition for an extended period while investigations and drying operations were arranged.

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Most of the contents and materials were present in this environment for around three months, with *some remaining for up to five months*, until the final strip-out was completed toward the end of July 2025.

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Throughout this period, the house was subject to *persistent dampness and high humidity*, consistent with long-term *Category 3 (sewage)* contamination.

Whilst we acknowledge that a period of drying and treatment was undertaken following the first strip-out, this was stopped when it became clear that a further strip-out would be required. No drying or dehumidification has been undertaken following the second strip-out, to the best of my knowledge. During both strip-outs, there was an evident stench of sewage and high levels of moisture were apparent, conditions which the builders are able to attest to.

At our most recent meeting with *our expert, Mr Adam Clutterbuck, and the insurance surveyor*, it was observed by Mr Clutterbuck that *“visible wet areas were evident along the floor-slab edges and at joints between sections of the slab.”* This observation was made during a prolonged dry period and therefore indicates *ongoing moisture ingress within the building fabric, providing continuing conditions for microbial persistence and activity.*

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2. Medical Backdrop (recorded for context, not for agreement)

It is relevant to record the professional opinion of *Dr Vanya Gant*, Consultant Physician in Cellular Immunology and Infectious Diseases, who has been directly involved in Mrs Louisa Scarr’s care. Dr Gant has confirmed that:

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Mrs Scarr is *clinically vulnerable*, having undergone major surgery, radiotherapy for breast cancer, and now suffering from *advanced chronic kidney disease* while carrying a *multidrug-resistant Gram-negative organism*.

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This combination places her at a *significantly heightened risk of both bacterial and fungal infection*, the latter occurring through inhalation of airborne mould spores which may also trigger hypersensitivity reactions.

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Dr Gant describes sewage as *“one of the richest sources of both bacteria and moulds known to man,”* noting that residual dampness will hugely increase airborne mould levels and that it is therefore *“very important for the preservation of Mrs Scarr’s future health to make every effort to reverse the totality of the sewage-related damage … including double deep cleaning to hospital standards.”*

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3. Airborne Contamination and Implications for Materials and Equipment

The period during which the property and its contents remained in a sewage-contaminated, high-humidity environment resulted in *airborne dissemination of microbial material throughout both floors of the house.* Porous substrates — particularly kitchen units, timber joinery, plaster surfaces, insulation, soft furnishings, *wooden furniture*, and *canvases* — are prone to absorbing and retaining these airborne contaminants.

In addition, *electrically ventilated or air-handling appliances* — including extractor fans, cooker hoods, washing machines, dishwashers, consumer units, refrigerators, *Everhot cookers*, and televisions with cooling vents — would have drawn contaminated air directly through their housings and internal components. These systems cannot be effectively cleaned or validated as sterile.

While in theory certain forms of specialist sterilisation exist for industrial or laboratory-grade equipment, *such processes are entirely impractical and economically prohibitive in a domestic context.* No commercially realistic method exists to decontaminate domestic appliances to a standard approaching hospital sterility.

Given the nature and duration of the exposure, these materials and systems *cannot
be decontaminated to a standard consistent with hospital-grade sterility* and must therefore be *removed and replaced.*

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4. Points Confirmed During Our Meeting

During our discussion, you confirmed that:

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There is *no economically viable method* to verify the complete elimination of pathogens from porous or absorbent materials once exposed to sewage.

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Surface or ATP swab testing is limited to the exact area and time tested and *cannot evidence systemic sterility.*

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For completeness, I note that ATP surface testing is sometimes carried out in such situations; however, as was discussed, such testing can only ever provide a momentary indication at the precise point sampled and cannot demonstrate systemic sterility or the absence of embedded contamination.

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Any attempt to restore such materials to hospital standards would be *scientifically unreliable outside the context of a laboratory sanitisation programme costing far in excess of any practical or economic threshold.*

On that basis, and from a biotechnical perspective, it is clear that if the objective is to return the property to a hospital-grade standard, it would be impossible to disagree with the following statement:

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Porous and absorbent materials, including kitchen units, joinery, plasterboard/plaster, insulation, timbers, soft furnishings, *wooden furniture*, and *canvases*, must be removed and replaced.

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Electrically ventilated or air-handling appliances, including extractor fans, cooker hoods, washing machines, dishwashers, consumer units, refrigerators, *Everhot cookers*, and televisions with cooling vents, must also be replaced.

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Full internal redecoration is required throughout to address the prolonged airborne exposure and humidity affecting both ground and first floors.

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5. Clarification of Shared Understanding

Many of the points above have already been rehearsed between us on several occasions, and I recognise that they already reflect BioResponse’s own professional view. The purpose of this record is to remove any possible ambiguity about that position. Both Andy and I recall, for example, your remark that you were surprised the kitchen units had not already been classified as Beyond Economic Repair (BER). In the wider context of our discussions, the same principle clearly extends to all other categories

listed. While the decision on replacement ultimately rests with insurers, this letter I trust stands as an accurate and fair record of the biotechnical position that we would both agree upon.

I would be grateful if you could confirm in writing that this summary reflects BioResponse's professional understanding of the points raised during our discussion.

Many thanks Darren.

Nicholas

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